

DEXASONE ELIXIR 0.5MG/5ML

Each 5ml contains:-
Dexamethasone 0.5mg

Product Description:
Clear, yellow syrup with pineapple flavour.

Pharmacodynamics:
Dexamethasone has the capacity to prevent or suppress the development of the local heat, redness, swelling and tenderness by which inflammation is recognized at the gross level of observation. At the microscopic level, they inhibit not only the early phenomena of the inflammatory process (edema, fibrin deposition, capillary dilatation, migration of phagocytes into the inflamed area, and phagocytic activity) but also the later manifestations (capillary proliferation, fibroblast proliferation, deposition of collagen and still later cicatrization). Naturally occurring glucocorticoids which also have salt-retaining properties, are used as replacement therapy in adrenocortical deficiency states. Their synthetic analogs including dexamethasone are primarily used for their potent anti-inflammatory effects in disorders of many organ systems. Glucocorticoids cause profound and varied metabolic effects. In addition, they modify the body's immune responses to diverse stimuli. At equipotent anti-inflammatory doses, dexamethasone almost completely lacks the sodium-retaining property of hydrocortisone. It is among the most potent glucocorticoids, having about 25-30 times the anti-inflammatory activity of hydrocortisone. In contrast, its effect on electrolytes is slight.

Pharmacokinetics:
Dexamethasone is effective when given by mouth and is absorbed from the gastrointestinal tract. The steroid exists in plasma in two forms: a protein-bound form and free form. The binding is accounted for by two protein fractions. One, "corticosteroid-binding globulin" is a glycoprotein and the other is plasma albumin. The globulin has high affinity but low total binding capacity; the albumin has low affinity but relatively large binding capacity. Consequently, at low or normal concentration of corticosteroids most of the hormone is bound to globulin. When the amount of corticosteroid is increased, concentration of both free and albumin-bound steroid increase with little change in the concentration of that bound to the globulin.

Indication:
It is indicated in endocrine disorders, rheumatic disorder, collagen diseases, dermatologic diseases, allergic states, ophthalmic diseases, respiratory diseases, hematologic disorders, neoplastic diseases, edematous states and gastrointestinal diseases.

Recommended Dosage:
5ml to 90 ml daily in divided doses. Dosage requirements are variable and must be individualized on the basis of the disease and the response of the patient.

Route of Administration: Oral

Contraindications:
Systemic fungal infection. Hypersensitivity to this product. Administration of live virus vaccinations.

Warnings and Precautions:
It should be used with caution in congestive heart failure, diabetes mellitus, chronic renal failure, hypertension, recent intestinal anastomoses, infectious diseases, myasthenia gravis, ocular herpes simplex, pregnancy and the elderly. Patients are warned to avoid exposure to chicken pox or measles. Patients with quiescent tuberculosis should be observed closely and should receive chemoprophylaxis if corticosteroid therapy is prolonged. Drug-induced secondary adrenocortical insufficiency may result from too rapid withdrawal of corticosteroids and may be minimized by gradual reduction of dosage. Corticosteroids may mask some signs of infection, and new infection may appear during their use. Prolonged use of corticosteroids may produce posterior subcapsular cataracts and glaucoma with possible damage to the

optic nerves and may enhance the establishment of secondary ocular infections due to fungi or viruses. Following prolonged therapy, withdrawal of corticosteroids may result in symptoms of the corticosteroids withdrawal syndrome including fever, myalgia, arthralgia and malaise. This may occur in patients even without evidence of adrenal insufficiency. There is an enhanced effect of corticosteroids in patients with hypothyroidism and in those with cirrhosis.

Interactions with Other Medicaments:
Aspirin should be used cautiously in conjunction with corticosteroids in hypoprothrombinemia. Phenytoin, phenobarbital, ephedrine and rifampicin in decreased blood levels and lessened physiologic activity, thus requiring adjustment in corticosteroid dosage. These interactions may interfere with dexamethasone suppression tests which should be interpreted with caution during administration of these drugs. The prothrombin time should be checked frequently in patients who are receiving corticosteroids and coumarin anti-coagulants at the same time because of reports that corticosteroids have altered the response to these anti-coagulants. It has been shown that the usual effect produced by adding corticosteroids is inhibition of response to coumarins. When corticosteroids are administered concomitantly with potassium depleting diuretics, patients should be observed closely for development of hypokalemia.

Co-treatment with CYP3A inhibitors, including cobicistat-containing products, is expected to increase the risk of systemic side-effects. The combination should be avoided unless the benefit outweighs the increased risk of systemic corticosteroid side-effects, in which case patients should be monitored for systemic corticosteroid side-effects.

Pregnancy and Lactation:
Since human reproduction studies have not been done with corticosteroids, use of these drugs in pregnancy or in women of childbearing potential requires that the anticipated benefits be weighed against the possible hazards to the mother and embryo or fetus. Infants born of mother who have received substantial doses of corticosteroids during pregnancy should be carefully observed for sign of hypoadrenalism. Corticosteroids appear in breast milk and could suppress growth, interfere with endogenous corticosteroid production, or cause other unwanted effects. Mothers taking pharmacologic doses of corticosteroids should be advised not to nurse.

Side Effects:
Side effects include fluid and electrolyte disturbances, muscle weakness, edema, hypertension, osteoporosis, acute pancreatitis, amenorrhea, Cushingoid state, growth retardation, mental disturbances, increased intraocular pressure, visual disturbances, local atrophy and increased appetite.

Symptoms and Treatment of Overdose:
The toxic effects of overdosage should be treated symptomatically and dosage reduced or the drug slowly withdrawn. Sodium intake needs to be reduced to less than 1g daily and potassium supplements may be necessary.

Effects on Ability to Drive and Use Machine: None known.

Storage Conditions: Store at or below 30°C. Protect from light.

Pack Sizes: A bottle of 60ml, 100ml and 120ml.
Pack Sizes (export only): A bottle of 1 litre and 3.6 litres.

Shelf-life: 1.5 years.
FURTHER INFORMATION CONCERNING THIS DRUG CAN BE OBTAINED FROM YOUR FAMILY PHYSICIAN / LOCAL GENERAL PRACTITIONER / PHARMACIST.

Manufacturer & Product Registration Holder:
Sunward Pharmaceutical Sdn. Bhd.
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81200 Johor Bahru, Johor, Malaysia.

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